

NorCal Imaging Center

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PATIENT: Park, David
DOB: 09/24/1986
SEX: M
ACCESSION: NCI-2024-99821
ORDERING MD: Reza Tehrani, MD
EXAM DATE: 07/01/2024
REPORTED: 07/01/2024 16:48 PT

EXAMINATION: MRI LUMBAR SPINE WITHOUT CONTRAST

CLINICAL INDICATION

Right-sided low back pain with radicular symptoms in L5/S1 distribution. Persistent for approximately 7 weeks despite conservative management. Evaluate for disc herniation or other structural pathology.

TECHNIQUE

Multi-planar, multi-sequence MR imaging of the lumbar spine was performed at 3.0 Tesla using sagittal T1, sagittal T2, sagittal STIR, axial T2, and axial T1 sequences. No intravenous contrast was administered.

COMPARISON

None available.

FINDINGS

ALIGNMENT: Normal lumbar lordosis is preserved. No spondylolisthesis. Vertebral body heights are maintained. No acute compression deformity. Bone marrow signal is normal.

CONUS MEDULLARIS: Terminates at the L1 level, normal in position and signal.

PARASPINAL SOFT TISSUES: Unremarkable. No paraspinal mass or fluid collection.

DISC-LEVEL FINDINGS:

- L1-L2: Disc height and signal preserved. No disc bulge or herniation. Neural foramina patent. Spinal canal normal in caliber.
- L2-L3: Disc height and signal preserved. No significant disc pathology. Neural foramina patent.
- L3-L4: Mild disc desiccation without loss of disc height. Small posterior disc bulge without focal protrusion. Neural foramina patent bilaterally. Spinal canal normal.
- L4-L5: Moderate disc desiccation with mild loss of disc height. There is a 5 mm right paracentral disc protrusion extending into the right lateral recess and abutting the traversing right L5 nerve

root. Mild-to-moderate right neural foraminal narrowing. The left neural foramen is patent. Mild ligamentum flavum thickening contributes minimally to central canal narrowing; the central canal remains adequate.

L5-S1: Mild disc desiccation. Small broad-based posterior disc bulge without focal protrusion. Mild bilateral facet arthropathy. Neural foramina patent bilaterally. The descending right S1 nerve root is not clearly contacted.

FACET JOINTS: Mild facet arthropathy at L4-L5 and L5-S1 bilaterally.

SACROILIAC JOINTS: Unremarkable.

IMPRESSION

1. 5 mm right paracentral disc protrusion at L4-L5 with impingement on the right L5 nerve root and mild-to-moderate right neural foraminal narrowing. This finding correlates with the patient's reported right-sided radicular symptoms.
2. Mild multilevel degenerative disc disease, most prominent at L4-L5.
3. No high-grade central canal stenosis. No acute fracture. No suspicious marrow lesion.

Findings discussed with the ordering provider's office at 17:02 on the date of the exam.

Electronically signed by:

Priya Krishnan, MD

Diagnostic Radiology, fellowship-trained MSK

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